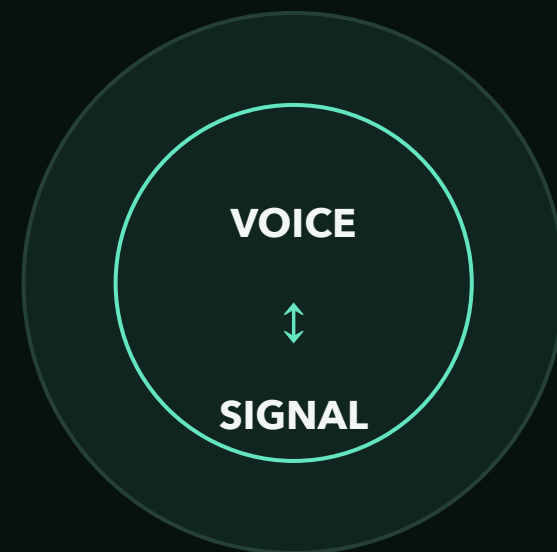




DUBAI ANGEL INVESTORS • FUNDING APPLICATION

Closed-loop voice AI for mental health support

A structured voice session that can observe the body's response – and know when to stop.



screen • adapt • verify

Andrey Kuzmin

Co-founder & CEO

neuroaloud.com

Consumer digital mental health

2,113 people used the voice product without a dollar of marketing. FOUNDER-REPORTED • AS OF 26 JULY 2026

2,113

total users to date

All current users are in Russia, using the product in Russian. Payments are off; revenue is \$0.

Almost all

sessions are voice

\$0

marketing spend

~70%

return for another session*

50+

largest age segment

What this proves

organic demand for voice

What it does not prove

U.S. willingness to pay, CAC or renewal

*Behavioral observation, not a defined retention metric.

Since early 2026: one working product, two working prototypes.

Only dated facts. Where we have nothing – press, awards, partnerships – we say so.

EARLY 2026

Work on Neuro begins
in its current form

26 JUL 2026

2,113 total users
zero marketing spend

BY SEP 2026

Two working
prototypes
none with users yet

05 SEP 2026

One Mind Accelerator
application submitted

SEP 2026

Both founders commit
to relocating to Dubai

No press, no awards, no partnerships to report. The 500-person cohort referenced elsewhere sits inside these users – it was not a separate study.

Expertise behind it: protocol written by a practising clinician with 20+ years · CTO with 3 patents and 35+ shipped hardware projects · CEO who built an engineering company to 100+ people

A large market in dollars. Our own slice: honestly, not yet.

Industry estimates below are third-party research, and they disagree with each other. We show the range.

\$8.97B

US digital mental health, 2026

towardshealthcare estimate;
~20% CAGR projected

\$8.6-9.6B

global mental health apps, 2026

range across Fortune BI,
GM Insights, DataM

\$931M

UAE digital health, 2025

Precedence Research;
~23% CAGR projected

WHO WE SERVE TODAY, AND OUR NICHE

Today: Russian-speaking adults seeking support between or instead of specialist visits; the largest group is 50+, which happened on its own. The niche: clinician-designed voice sessions plus a wearable that changes the session live. No consumer EEG headband carries EDA; Ash, Sonia and Wysa hold conversations but record no synchronized physiology.

WHAT WE WILL NOT CLAIM

We have no dollar figure for our own obtainable share. It cannot exist before US conversion, CAC and renewal data exist. Any number we printed here today would be invention.

The product started with a boundary problem – not a chatbot idea. The 500 are users inside Neuro, counted in the 2,113.

500

of our first users – participants, not a separate study

Led by Olga Sagalaeva, Clinical Lead
20+ years in practice

NOT A FORMAL CLINICAL STUDY

The question

Can people safely work through real psychological problems with general-purpose AI?



The finding

The central failure was not usefulness. It was boundary awareness.



The build

A protocol with screening, refusal and escalation.

A general chatbot is optimized to continue. In mental health support, the safer action may be to stop.

Words reveal the story. They do not reveal the full state.

Most digital mental-health support must infer whether the user is calming, freezing or escalating from self-report alone.

"I'm fine."

The system still has to guess:

continue • slow down • pause • stop

29.3M

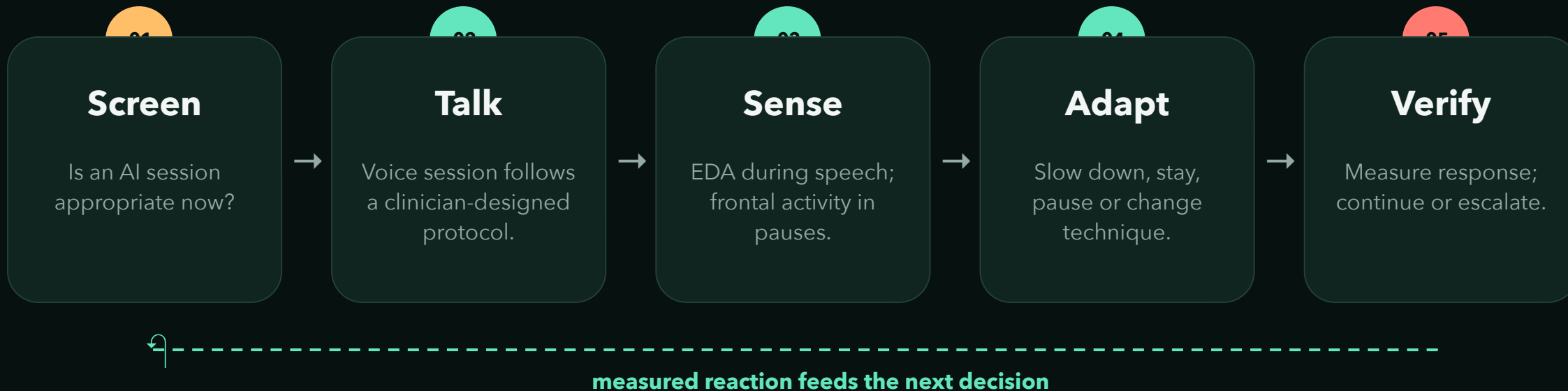
**U.S. adults with a mental illness
received no mental-health
treatment in 2022**

Calculated from NIMH: 59.3M with AMI minus 30.0M who received treatment.

Neuro's thesis: access matters, but observability and boundaries determine whether AI support can be trusted.

One continuous loop – inside the session.

The physiological signal is not another dashboard. It is an input that changes what the system does next.



sensing only • no stimulation

Live today: https://t.me/neuro_psylga_bot (Russian-language) • neuroaloud.com

Conversation creates noise. The sensing design starts there.



WORKING PROTOTYPE • NOT A RENDER

DURING SPEECH

EDA is primary

Jaw and facial muscle artifacts contaminate frontal electrical sensing while a person talks.

IN PAUSES + GUIDED EXERCISES

Frontal activity + muscle tone

SENSING ONLY • NO STIMULATION

DRY

frontal electrodes

EDA

finger sensor

70-80 g

total weight

BLE

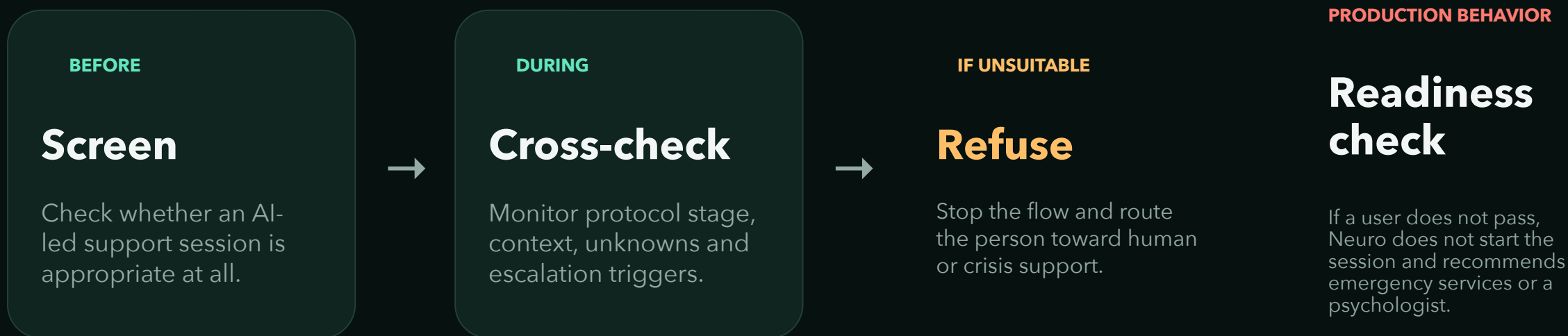
wireless link

TIMED

at acquisition

The system must know when not to run a session.

Refusal is a product behavior – not a disclaimer at the bottom of the screen.



The safety question is continuous:

Should the system continue, change course, stop, or escalate?

One membership. Two prices. Three unknowns to resolve.

FREE

\$0

3 voice sessions / month
No device

Acquire through product value

COMPLETE

\$249 / year

Unlimited sessions
Sensing device included

One outcome – not hardware + software

PROPOSED • PAYMENTS NOT ENABLED

THE FIRST PAID COHORT MUST MEASURE

- 01 CAC**
What does trusted acquisition cost?
- 02 Renewal**
Will people keep the annual plan?
- 03 Signal lift**
Does physiology improve a defined outcome?

DEVICE BOM

\$80 @ 100

\$60 @ 1K

\$28 @ 10K

Gross margin is modelled on stated assumptions – see the financials slide. Nothing here is proven by paid sales.

One channel is proven. The rest is a plan, not a result.

2,113 users arrived with zero marketing spend. That is not a low CAC – it is no CAC, and no CAC data.

THE SEQUENCE WE WILL RUN

01 Brand platform

12 archetypes – decide what the product is to a person before buying traffic

02 Visual identity

built on that platform

03 Channel tests

social, creators, paid search – tests, not declarations

WHAT WE ARE NOT CLAIMING

The first US segment is not chosen. Users over 50 are our largest group, but that happened on its own – we do not present it as a targeting decision.

No partner, no pilot, no budget, no dates. In the region: relocation is decided and we intend to apply to the Dubai health data sandbox. Nothing there is signed.

WHAT WE MEASURE IN YEAR ONE

Cost of acquisition on a paid market · annual renewal rate · whether the physiological signal adds measurable outcome. None of the three is known today, and we do not estimate them.

Clinical judgment, full-stack engineering and company building – in one team.



Dmitriy Kareisha

CTO • co-founder

Andrey Kuzmin

CEO • co-founder

Olga Sagalaeva

Clinical Lead
20+ years in practice
Protocol, measurements,
safety

Dmitriy Kareisha

CTO + co-founder
3 prior patents
35+ shipped hardware
projects

Andrey Kuzmin

CEO + co-founder
Built an engineering
company to 100+ people
Product, GTM, fundraising

Yulia Litvinova

Product & Creative
US permanent resident in
California
Design and content

Founders: Andrey + Dmitry • Olga and Yulia are core team, not founders • Not yet incorporated

No advisors and no outside investors – founder-funded. The round hires device engineering and a first growth lead.

Five-year projection, and what happens if we are wrong.

Projection on stated assumptions, not a forecast we can evidence. Every assumption is named below.

YEAR	CUSTOMERS	REVENUE	GROSS PROFIT	EXPENSES	EBITDA	MARGIN
1	1,000	\$249K	\$150K	\$450K	-\$300K	60%
2	6,000	\$1.49M	\$1.05M	\$1.35M	-\$300K	70%
3	20,000	\$4.98M	\$4.09M	\$3.30M	+\$788K	82%
4	50,000	\$12.45M	\$10.26M	\$6.80M	+\$3.46M	82%
5	100,000	\$24.90M	\$20.60M	\$11.40M	+\$9.20M	83%

EBITDA POSITIVE IN YEAR 3

STRESS TEST – EBITDA YEAR 3

base case +\$788K · renewals at 50% +\$755K
renewals 50% AND AI cost doubled +\$456K
Still positive in year 3 under the worst case.

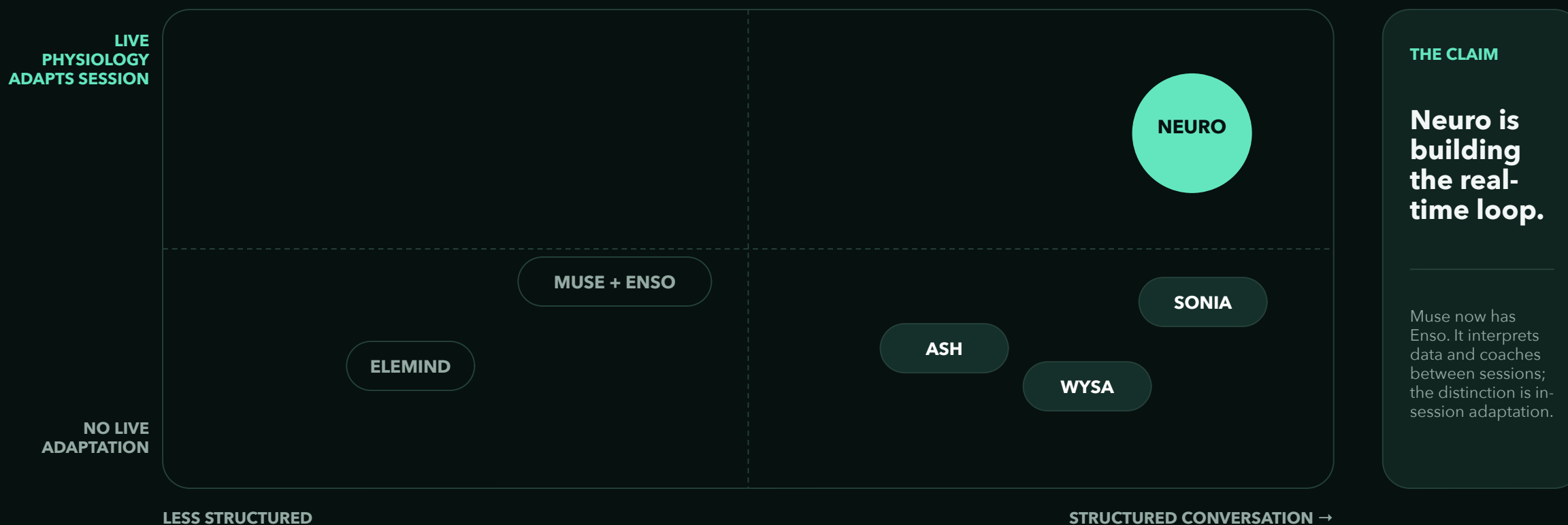
ASSUMPTIONS, NAMED

Subscriber growth and 70% renewal are assumptions, not observed – the 70% comes from free sessions in Russia, not paid renewals. AI cost per user is unevidenced; modelled at \$23 and at \$46. We state no LTV, free-to-paid conversion or market penetration – nobody has paid us yet.

Capex on top: \$50 / 80 / 150 / 250 / 350K by year. Stated inputs: \$249 is a planned price with payments off · BOM \$80 / \$60 / \$28 are batch quotes · the only verified figure is card processing at \$7.52 · year 1 costed at the 100-unit price on purpose.

The open space is not 'AI + wearable.' It is live adaptation.

Who changes the session while it is happening, based on synchronized physiological response?



We have not proven that physiology improves the result.

So the next milestone is a falsifiable comparison – not a larger claim.

ARM A

Signal reaches the model

Same session protocol
Live physiological input
Adaptation based on the signal

VS

ARM B

Signal is withheld

Same session protocol
Sensor records silently
Model cannot use the signal

If signal adds value

→ **quantify effect and design the evidence path**

If it does not

→ **the voice product still stands on observed demand**

Outcomes and retention definitions must be set before enrollment.

Raising \$500,000 at \$4M pre-money.

No outside capital raised to date. Founder-funded.

\$500K

round size

minimum \$300K · maximum \$750K

\$4M

pre-money valuation

minimum ticket \$25,000

12-17 mo

runway this buys

12 months if year-one revenue does not arrive, ~17 if the model holds. Not to profitability.

USE OF PROCEEDS – OF THE \$500K RAISED

- 25%** Team and engineering – device to production
- 22%** Devices, tooling, test equipment (capex)
- 18%** UAE pilot and the evidence work
- 15%** Sales and marketing – first paid market
- 15%** Founder salaries – two people, modest
- 5%** Legal, incorporation, regulatory

INSTRUMENT AND TIMING

Equity. The exact form – priced round or SAFE – is settled with the lead investor once the Delaware C-Corp exists. The company is not incorporated yet and we do not hide that.

Target: close within 3-4 months of first meeting.

Existing investors: none.

We cannot tick your fifth box. Here is what we bring instead.

Not active in MENA today: every user is in Russia. We answer No and do not dress it up.

WHY THE UAE SPECIFICALLY

Both founders are relocating to Dubai – decided.

Dubai's Health Data Sandbox (Dubai Future Foundation with the Dubai Health Authority) is a legal route to test a sensing device and a clinician-designed protocol against de-identified health data and a regulator. US state law passed in 2025-26 made that path materially harder for this category.

We intend to apply. We do not hold access today.

WHAT WE NEED FROM DAI BEYOND MONEY

A sponsor on the Investment Committee to carry this forward.

Introductions to clinics, employers and partners in the region for a first pilot.

Co-investors for the round.

Regulatory know-how in the UAE.

WHAT WE BRING

working voice product

working sensing
prototype

clinician-designed
protocol

2,113 users, zero
marketing

two founders, full time,
moving to the region